

Pulmonology Service Line Handbook

MERIDIAN HEALTH PARTNERS
OPERATIONS POLICY MANUAL

DEPARTMENT: Pulmonology
SECTION: Service Line Handbook
EFFECTIVE DATE: 2024-03-15
REVISION: 1.0

1.0 INTRODUCTION

1.1 This document outlines the operational policies and service scope for the Pulmonology Department across all fourteen (14) Meridian Health Partners outpatient clinics.

1.2 Adherence to these guidelines ensures consistent patient care delivery and clarifies departmental responsibilities.

1.3 This handbook does not include diagnostic or treatment guidance.

2.0 CONDITIONS ACCEPTED FOR CONSULTATION AND MANAGEMENT

2.1 The Pulmonology Department accepts referrals for the following conditions:

2.1.1 Asthma (controlled and uncontrolled).

2.1.2 Chronic Obstructive Pulmonary Disease (COPD), including emphysema and chronic bronchitis.

2.1.3 Chronic cough, persistent for eight (8) weeks or longer, without acute infectious cause.

2.1.4 Interstitial Lung Disease (ILD), including idiopathic pulmonary fibrosis and sarcoidosis.

2.1.5 Obstructive Sleep Apnea (OSA) for initial diagnostic workup interpretation and ongoing management.

2.1.6 Bronchiectasis.

2.1.7 Post-pneumonia follow-up, minimum four (4) weeks post-acute infection resolution.

2.1.8 Pleurisy and stable, non-malignant pleural effusions.

2.1.9 Unexplained dyspnea after initial cardiac workup exclusion.

2.1.10 Suspected occupational lung diseases.

2.1.11 Alpha-1 antitrypsin deficiency (initial screening and management).

3.0 CONDITIONS NOT ACCEPTED FOR INITIAL CONSULTATION OR PRIMARY MANAGEMENT

3.1 The Pulmonology Department does not accept direct referrals for the following conditions, requiring initial management by other specialists or acute care settings:

3.1.1 Active Tuberculosis (TB): Refer to local public health infectious disease services.

3.1.2 Acute Respiratory Distress Syndrome (ARDS): Requires inpatient critical care management. Refer to emergency department or hospital critical care services.

3.1.3 Active Lung Cancer Diagnosis or Staging: Refer to Meridian Oncology. The Pulmonology Department will accept referrals for *pulmonary symptom management* in established lung cancer patients only upon direct referral from a Meridian Oncology physician.

3.1.4 Acute Pulmonary Embolism (PE): Requires emergency department assessment and acute inpatient management. Follow-up consultation for long-term management is accepted *after* initial stabilization and discharge from acute care.

3.1.5 Cystic Fibrosis (CF): Refer to specialized CF centers (e.g., St. Jude's Medical Center CF Clinic). The department will perform initial screening spirometry for suspected CF if ordered by a pediatrician, but ongoing management is outside the scope of our general pulmonology services.

4.0 TYPICAL APPOINTMENT DURATION

4.1 Initial Patient Consultations: Forty-five (45) minutes.

4.2 Follow-up Appointments: Twenty (20) minutes.

4.3 Pulmonary Function Testing (PFT) with Spirometry: Sixty (60) minutes.

4.4 Bronchodilator Challenge Test: Thirty (30) minutes.

5.0 PATIENT PREPARATION INSTRUCTIONS

5.1 For all initial consultations:

5.1.1 Patients must bring a comprehensive list of all current medications, including dosage and frequency.

5.1.2 Patients should bring relevant prior imaging reports (e.g., chest X-rays, CT scans) and laboratory results if these were performed outside the Meridian Health Partners network and not already forwarded.

5.2 For Pulmonary Function Testing (PFT) and Spirometry:

5.2.1 Patients must refrain from using short-acting bronchodilators (e.g., albuterol) for at least six (6) hours prior to the appointment.

5.2.2 Patients must refrain from using long-acting bronchodilators (e.g., salmeterol, tiotropium) for at least twelve (12) hours prior to the appointment.

5.2.3 Patients should avoid heavy meals for two (2) hours prior to testing.

5.2.4 Patients should wear loose, comfortable clothing to allow for full chest and abdominal expansion.

5.3 For Sleep Study Interpretations:

5.3.1 Patients must bring the completed sleep diary (if provided by the referring physician) and any home sleep study device data.

6.0 MERIDIAN HEALTH PARTNERS INSURANCE PLAN COVERAGE FOR PULMONOLOGY SERVICES

6.1 Meridian Basic Plan: Covers initial consultations, follow-up appointments, basic spirometry (CPT 94010), and pulse oximetry (CPT 94760).

6.2 Meridian Core Plan: Covers all services under Meridian Basic, plus comprehensive Pulmonary Function Testing (PFTs, CPT 94060, 94070), bronchodilator challenge tests (CPT 94060 add-on), and initial interpretation of home sleep studies (CPT 95806).

6.3 Meridian Premium Plan: Covers all services under Meridian Core, plus in-clinic nebulizer treatments (CPT 94640), specialized consultations for complex cases, and advanced imaging interpretation (e.g., high-resolution CT chest interpretation).

6.4 Meridian Executive Plan: Covers all services under Meridian Premium, with the added benefit of expedited scheduling (guaranteed appointment within five (5) business days of request) and dedicated specialist coordination services managed by Ms. Eleanor Vance, Senior Patient Liaison.

7.0 REFERRAL TURNAROUND COMMITMENT

7.1 Standard Referrals: All non-urgent referrals received by the Pulmonology Department will be processed, and the patient contacted for scheduling, within three (3) business days of receipt.

7.2 Urgent Referrals: Referrals explicitly marked "URGENT" by the referring physician, or those indicating acute exacerbation of COPD, uncontrolled asthma, or new onset significant dyspnea, will be processed and the patient contacted within one (1) business day of receipt. The referring physician will be notified via secure message (MeridianMD Portal) of the scheduled appointment time within this period.

8.0 CONTACT INFORMATION

8.1 Department Head: Dr. Aris Thorne

8.2 Clinical Coordinator: Mr. Ben Carter

8.3 General Inquiries: Ext. 4500